CASE PRESENTATION

MELWIN MABEN
FINAL YEAR, HIMS HASSAN

Name: Mrs. XYZ

Age: 24 years

Sex: Female

Address: Chikkamagalur

Occupation: Housewife

Education: 10th Std.

Husband's Details: Mr. ABC, 27 years, Labourer, Educated up to 7th Std.

Socioeconomic status: Class 4 of Modified BG Prasad Classification

DOA: 05th March 2020

DOE: 07th March 2020

LMP: 20th June 2019

EDD: 27th March 2020

Period of Gestation: 37 weeks

Obstetric Score: $G_1P_0L_0A_0$

CHIEF COMPLAINTS

H/o Amenorrhea since 8 ½ months

C/o Bleeding per vagina 2 days ago

HISTORY OF PRESENTING ILLNESS

Patient was apparently alright 2 days. Then she developed bleeding per vagina which was sudden in onset, which completely soaks 1 pad, associated with bright red coloured blood. It stopped spontaneously.

The fetal movements are well percieved

- No H/o pain in abdomen associated with bleeding
- No H/o excessive fatiguability, palpitations
- No H/o blurring of vision or pedal edema
- No H/o Trauma
- No H/o Leaking per Vagina

OBSTETRIC HISTORY

Married Life: 4 years

Non Consanguineous marriage

Obstetric Score: $G_1P_0L_0A_0$

HISTORY OF PRESENT PREGNANCY

First Trimester:

-Spontaneous Conception

-Pregnancy was confirmed after 1 ½ months of missed period by UPT and she was a booked case at HIMS.

-3 ANC visits completed

-Folic Acid tablets were taken

-First Trimester (Dating Scan) was done and was found normal and corresponding to gestational age

No h/o burning micturition.

No h/o fever with rashes, excessive vomiting.

No h/o spotting or bleeding per vagina.

No h/o pain abdomen.

No h/o exposure to radiation and drug intake.

- **Second Trimester:**

-Quickening was felt at 5 ½ months of gestation

-Second Trimester(Anomaly Scan) was done and no anomalies were found.

-Iron and Calcium Tablets were taken

-Two doses of TD vaccine taken

-No History of fever with rashes

-No History of any drug intake or radiation exposure

-No History of Per vaginal bleed

-No H/o Pedal edema or headache

-No H/o Raised BP Recording or Raised Blood Sugar recording

- **Third Trimester:**

-Fetal movements are well perceived

-Iron and Calcium tablets taken

-Growth scan done at 32 weeks and showed normal development

-Weight gain in pregnacy-12 kg

MENSTRUAL HISTORY

Menarche: 13 years

Past cycles:

28 days cycle	3-4 days of flow
2-3 pads/day	No H/o Dysmenorrhea, passage of clots

LMP: 20th June 2019

EDD: 27th March 2020

CONTRACEPTIVE HISTORY

Barrier Method (Condoms) were used as contraception

PAST HISTORY

Not a known case of Diabetes mellitus, Hypertension, Epilepsy, Asthma, TB, Thyroid Disorders.

No H/o Surgeries in the past

No H/o Previous Blood Transfusions

FAMILY HISTORY

-No history bleeding disorder in the family.

-No history of children with chromosomal anomaly /birth defect.

-No history of twining in the family.

PERSONAL HISTORY

- Diet: Mixed
- Appetite: Normal
- Sleep: Undisturbed
- Bowel and Bladder: Normal and regular
- No H/o any addictions

SUMMARY

A 24 year lady with an Obstetric Score of $G_1P_0L_0A_0$ with 8 ½ months of amenorrhea comes with one episode painless bleeding per vagina 2 days ago. It was sudden in onset. Fetal movements are well percieved.

GENERAL PHYSICAL EXAMINATION

- Here is a young lady who is moderately built and nourished. She is conscious, cooperative and well oriented to time, place and person.
- Weight – 67 kg
- Height – 158 cm
- BMI – 26.91 kg/m^2
- Pallor, Icterus, Clubbing, Cyanosis, Lymphadenopathy, Edema: Absent
- Breast, Spine, Thyroid: Normal

VITALS

- Pulse rate – 76 beats/min, regular rhythm, normal volume, normal character
- Blood pressure – 120/80 mmHg. Taken from left hand in sitting position
- Respiratory rate – 16 cycles/min, thoraco-abdominal type
- Temperature – 98.6 F

PER ABDOMEN EXAMINATION

- INSPECTION

Shape of the abdomen- Uniformly Distended

Flanks: Full

Umbilicus is central and everted.

Linea nigra and Stria gravidarum: Present.

No scars, sinuses or dilated veins can be seen.

Hernial orifices are intact.

- PALPATION

No local rise of temperature or tenderness.

Uterus is relaxed

Abdominal girth: 99 cm, at the level of umbilicus.

Fundal height: 36 weeks, corresponds to gestational age.

Symphysio-fundal height: 36 cm, corresponds to gestational age.

OBSTETRIC GRIPS :

Fundal grip : Broad, soft, irregular mass suggestive of breech.

Right Lateral grip : Uniform, continuous curved resistance suggestive of spine.

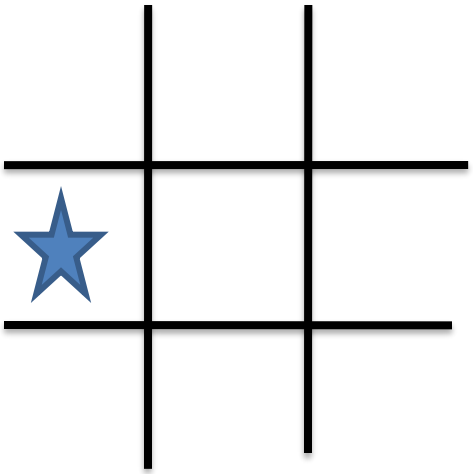
Left lateral grip : Multiple knob like structures suggestive of limbs.

1st Pelvic grip : Hard, ballotable mass suggestive of fetal head

2nd Pelvic grip : Fingers are converging.

AUSCULTATION :

FHS heard along the right spinoumbilical line, rate - 140/min



SYSTEMIC EXAMINATION

Cardiovascular system

S1, S2 sounds heard, no murmurs heard

Respiratory system

Normal vesicular sounds heard

No added sounds

Central nervous system

No focal neurological deficit

SUMMARY

- A 24 year lady with an Obstetric Score of $G_1P_0L_0A_0$ with 8 ½ months of amenorrhea comes with one episode painless bleeding per vagina 2 days ago. It was sudden in onset. Fetal movements are well percieved.
- Her vitals are normal and on per-abdominal examination, the symphysio-fundal height corresponds to the period of gestation. A single live fetus is palpated with hard ballotable mass on 1st Pelvic Grip suggestive of Cephalic Presentation. The fetal heart sounds are normal.

PROVISIONAL DIAGNOSIS

A 29 year old lady with obstetric index $G_0P_0L_0A_0$ with 37 weeks of gestation with live single intrauterine pregnancy with Cephalic Presentation with Antepartum Haemorrhage under evaluation.

MCQ 1-All of the following are features of Placenta Previa except:

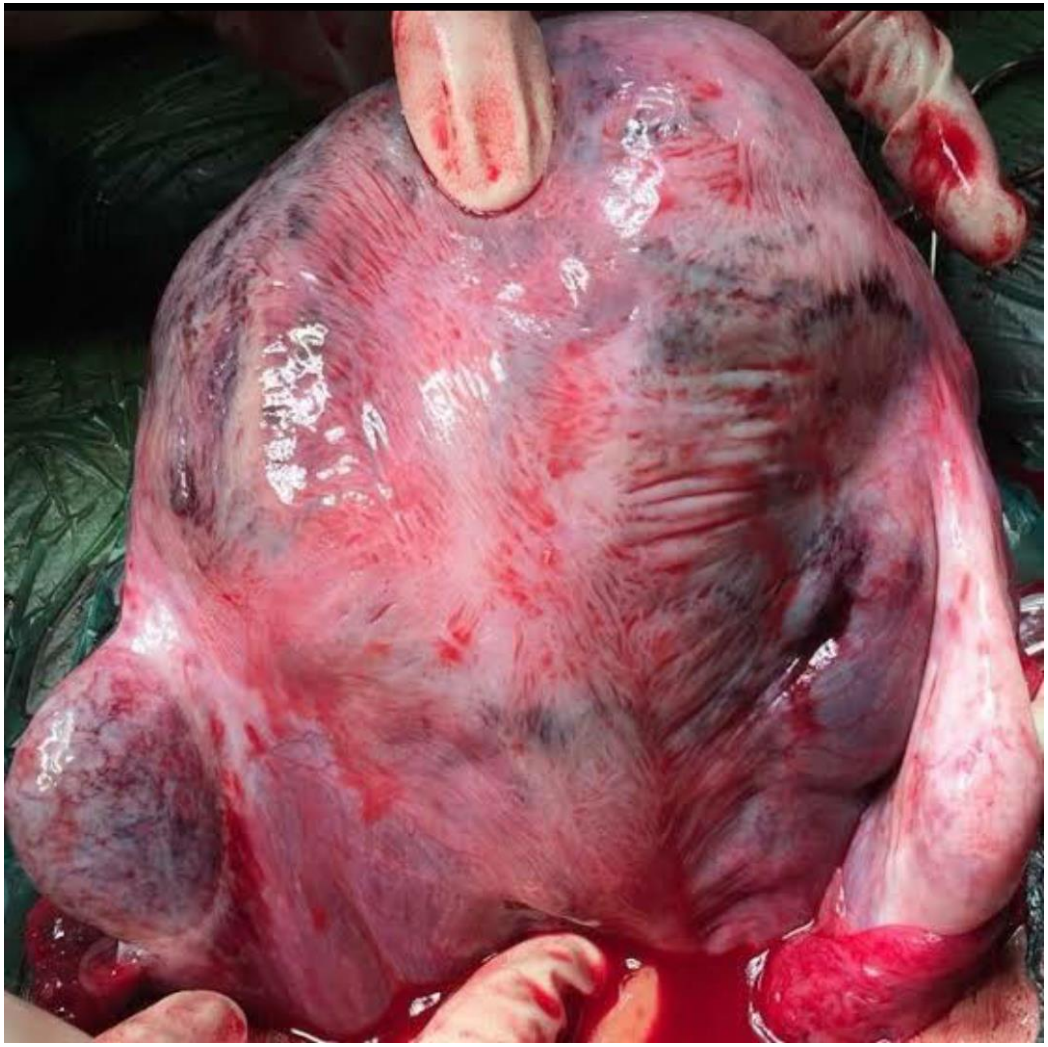
A) Painless, recurrent bleeding
B) Can be revealed or concealed type of bleeding
C) Height of Uterus is proprtional to gestational age
D) Uterus feels soft and relaxed

- Answer: B
- Bleeding is always revealed in Placenta Previa
- Ref: DC Dutta

Table 19.1: Distinguishing Features of Placenta Previa and Abruptio Placentae

Parameters	Placenta Previa	Abruptio Placentae
■ **Clinical features:**		
• Nature of bleeding	(a) Painless, apparently causeless and recurrent (b) Bleeding is always revealed	(a) Painful, often attributed to preeclampsia or trauma and continuous (b) Revealed, concealed or usually mixed
• Character of blood	Bright red	Dark colored
• General condition and anemia	Proportionate to visible blood loss	Out of proportion to the visible blood loss in concealed or mixed variety
• Features of preeclampsia	Not relevant	Present in one-third cases
■ **Abdominal examination:**		
• Height of uterus	Proportionate height to gestational age	May be disproportionately enlarged in concealed type
• Feel of uterus	Soft and relaxed	May be tense, tender and rigid
• Malpresentation	Malpresentation is common. The head is high floating	Unrelated, the head may be engaged
• FHS	Usually present	Usually absent especially in concealed type
■ **Placentography (USG)**	Placenta in lower segment	Placenta in upper segment
■ **Vaginal examination**	Placenta is felt on the lower segment	Placenta is not felt on lower segment. Blood clots should not be confused with placenta

2) Identify the Condition and name the cause for the same



- Ans: Couvelaire Uterus
- In concealed variety of Abruptio Placenta the blood enters the myometrium, uterus appears bluish or wine coloured.